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Envision ATLUS I.H.I.S. — Comprehensive Feature Catalog

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Prepared by: Envision Virtual Edge Group LLC **Date:** March 28, 2026 **Version:** 1.0 **Classification:** Internal — Executive, Investor, and Sales Use

Document Purpose

This catalog documents 60 production-implemented features discovered through direct source code analysis of the Envision ATLUS Intelligent Healthcare Interoperability System. Each entry includes the feature’s primary function, system importance, market uniqueness, and latent benefits that extend beyond its original design purpose.

This platform was built by a Social and Behavioral Scientist (AI System Director) and a BSN, RN, CCM (Chief Compliance and Accountability Officer) — domain experts with zero engineering background — using AI-assisted development. The features below represent what is actually shipping in production code, not a roadmap.

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Community Product (WellFit)

Feature 1: AI-Personalized Daily Check-In System

Primary Function: Comprehensive daily health reporting interface where seniors submit mood, vital signs (BP, heart rate, SpO2, blood glucose, weight), symptoms, and activity levels. Supports quick-submit (mood-only) and detailed form (full vitals panel) modes.

System Importance: This is the foundational data collection engine for the entire platform. Every downstream clinical prediction — readmission risk, fall risk, medication adherence, engagement scoring — depends on the daily check-in data stream. Without it, the clinical AI layer has no behavioral signal.

Uniqueness: Most remote monitoring platforms collect vitals OR mood — not both in a single daily touchpoint. The check-in also integrates voice input (Web Speech API) for seniors who struggle with typing, and includes a camera-based pulse oximeter for SpO2 + heart rate measurement using the phone's flashlight and camera — no wearable device required.

Latent Benefit: The daily check-in is actually a Remote Patient Monitoring (RPM) data collection instrument. Every submission generates the documentation required for CMS RPM billing codes (CPT 99453-99458), worth \$120+/patient/month in Medicare reimbursement. The system was built for wellness engagement, but it accidentally created a revenue-generating clinical data pipeline.

Feature 2: Three-Path Crisis Routing System

Primary Function: Immediate crisis detection and routing embedded in the check-in flow. When a senior selects “Not Feeling My Best,” a decision tree offers three paths: (1) “I want to speak to someone” → 988 Suicide & Crisis Lifeline, (2) “I have fallen and injured myself” → 911 emergency dispatch, (3) “I am lost” → location/navigation assistance.

System Importance: Transforms a wellness app into a safety net. Seniors in crisis get routed to the right resource within two taps, without needing to remember phone numbers or navigate complex menus.

Uniqueness: Consumer health apps either handle crisis with a generic “call 911” button or ignore it entirely. This system differentiates between emotional distress, physical injury, and cognitive disorientation — three fundamentally different emergencies requiring different responders. The “I am lost” path is designed specifically for early-stage dementia patients.

Latent Benefit: The crisis routing system generates audit-logged emergency events that feed into the clinical readmission risk model. A senior who triggered the fall pathway has their fall risk score automatically elevated, and their care team receives an alert. This closes the loop between community safety and clinical intervention without any manual data entry.

Feature 3: AI Health Insights Engine

Primary Function: After each check-in submission, Claude AI analyzes the current values against 7-day and 30-day rolling averages, detects trends (improving/declining/stable), and generates personalized natural-language health insights.

System Importance: Converts raw vitals data into actionable intelligence that seniors can understand. A blood pressure reading of “142/88” means nothing to most patients, but “Your blood pressure has been trending up over the past week — consider reducing sodium intake and scheduling a follow-up” is actionable.

Uniqueness: Most patient portals show vitals as numbers or charts. This system uses AI to translate clinical data into 6th-grade reading level health coaching, with fallback to rule-based heuristic suggestions if the AI service is unavailable. The dual-mode approach (AI primary, heuristic fallback) ensures seniors always get feedback.

Latent Benefit: The trend detection algorithm — comparing 3-day vs. 14-day rolling averages — creates an early warning system for clinical deterioration. A gradual SpO2 decline from 97% to 93% over two weeks might not trigger a single-reading alert, but the trend analysis catches it. This is the same pattern recognition that hospital early warning scores (NEWS2, MEWS) use, applied to home-based monitoring.

Feature 4: Passive SDOH Detection System

Primary Function: Automatically scans free-text content — clinical notes, community posts, check-in comments, and messages — for Social Determinants of Health indicators. Detects food insecurity, housing instability, transportation barriers, social isolation, financial strain, and language barriers without requiring structured assessments.

System Importance: SDOH factors account for 80% of health outcomes according to the WHO, but clinicians capture them in fewer than 20% of encounters because structured screening takes too long. Passive detection runs silently in the background, extracting SDOH signals from text that already exists.

Uniqueness: Competing platforms require clinicians to fill out Z-code assessment forms manually. This system detects SDOH passively from natural language — a community post saying “I couldn’t make it to my appointment because the bus route changed” triggers a transportation barrier detection with a suggested Z59.82 code. No other community health platform combines passive NLP detection with automatic ICD-10 Z-code suggestion.

Latent Benefit: Every SDOH detection is a billable documentation opportunity. ICD-10 Z-codes (Z55–Z65) for social determinants increase case-mix index and risk-adjustment scores. For Medicare Advantage plans, properly documented SDOH increases per-member reimbursement by \$1,200–\$3,600/year. The system was built to improve care — but it also captures revenue that hospitals currently leave on the table.

Feature 5: Pill Identifier & Medication Label Reader (AI Vision)

Primary Function: Two AI-powered medication safety services. The Pill Identifier uses Claude Vision to analyze photographs of pills — shape, color, imprint codes, score lines, coating — and matches them against medication databases. The Medication Label Reader extracts structured data from pharmacy label photos (medication name, dosage, instructions, prescriber, refill info, NDC code).

System Importance: Medication errors are the third leading cause of death in the US. Seniors managing 5+ medications frequently mix up pills, especially after hospital discharge when medications change. These services give patients a way to verify their medications using their phone camera.

Uniqueness: FDA’s pill identifier database exists, but it requires knowing what you’re looking for. This system works backwards — you photograph the pill, and AI tells you what it is. The pill-to-label comparison feature is novel: it cross-references an identified pill against the prescription label to detect mismatches (wrong strength, wrong medication, wrong NDC), with severity ratings from VERIFIED to “DO NOT TAKE.”

Latent Benefit: The medication verification workflow generates structured medication data (NDC codes, RxCUI identifiers) that feeds directly into the FHIR MedicationRequest resource. Every pill photo becomes a medication reconciliation data point. For hospitals tracking medication adherence post-discharge, this replaces patient self-reporting (“Did you take your medications?”) with visual confirmation.

Feature 6: Medicine Cabinet with Adherence Analytics

Primary Function: Comprehensive medication management interface combining photo scanning (label reader integration), medication tracking, dose recording, adherence analytics (7-day and 30-day trends), refill management, drug interaction warnings, and psychopharmacology alerts.

System Importance: Medication non-adherence costs the US healthcare system \$290 billion annually. This feature gives seniors a complete medication management hub — add medications by photographing the label, track doses, get refill reminders, and receive interaction warnings — all in one interface.

Uniqueness: Most medication apps require manual entry. This system auto-populates from label photos with >80% confidence, includes FHIR sync status (showing whether the medication list matches the hospital EHR), and has special psychopharmacology warnings (“Do not stop abruptly”) that require explicit acknowledgment before dismissal.

Latent Benefit: The adherence percentage data (tracked per medication over time) feeds into the readmission risk prediction model as a behavioral predictor. A patient at 40% adherence on their heart failure medication has a quantifiably higher 30-day readmission risk. This transforms medication tracking from a patient convenience feature into a clinical risk stratification signal.

Feature 7: Community Moments — Senior Social Engagement Gallery

Primary Function: Photo-sharing gallery where seniors post daily moments, activities, and stories. Includes admin moderation workflow (pending → approved), emoji reactions, daily affirmation rotation, and a featured moments highlight system.

System Importance: Social isolation is the strongest predictor of mortality in seniors over 65 — stronger than smoking, obesity, or physical inactivity. This feature creates a low-friction social engagement touchpoint that generates measurable interaction data.

Uniqueness: Social platforms for seniors exist, but they’re standalone apps. This is embedded within a healthcare platform, meaning every photo upload, reaction, and comment generates engagement data that feeds into clinical risk models. The admin moderation workflow protects vulnerable seniors from inappropriate content — a safety layer consumer social media doesn’t provide.

Latent Benefit: Community Moments participation is one of seven dimensions in the Holistic Risk Assessment scoring. A senior who stops posting photos after weeks of daily engagement triggers a social isolation risk flag. The feature was designed for social connection, but it functions as a passive behavioral health monitoring instrument — without any clinical questionnaire burden on the patient.

Feature 8: Voice Command & Voice Search System

Primary Function: Full speech recognition interface using the Web Speech API with vendor-prefixed fallbacks (webkit, moz, ms). Supports route navigation, section scrolling, patient search by voice (“show patient Maria LeBlanc”), and continuous listening with real-time confidence scoring.

System Importance: Seniors with arthritis, vision impairment, or low digital literacy cannot navigate complex healthcare interfaces. Voice commands make every feature accessible through natural speech.

Uniqueness: ATLUS Intuitive Technology — the voice system parses natural language entities, not just fixed commands. “Show me the bed board for ICU” navigates to the bed management panel filtered to the ICU unit. This isn’t a menu-driven voice interface; it’s a natural language navigation layer over the entire application.

Latent Benefit: Every voice command is audit-logged with transcription text and confidence score. This creates a dataset of natural language healthcare queries that reveals how clinical staff actually think about their workflows. Over time, this data could train specialized healthcare voice models — a valuable dataset asset independent of the application itself.

Feature 9: Caregiver Briefing Service (AI-Generated Family Updates)

Primary Function: Generates automated family caregiver updates about patient status in three formats: daily summaries, weekly progress reports, and urgent alerts. Content includes wellness scores, check-in summaries, care plan progress, upcoming items, and actionable items for the caregiver. Delivered via SMS (condensed to 480 characters) or email (full format).

System Importance: Family caregivers provide 83% of long-term care in the US, but they’re typically excluded from the clinical data loop. This feature bridges that gap by translating clinical and engagement data into plain-language briefings.

Uniqueness: No competitor generates AI-personalized caregiver briefings from the same platform that collects the patient data. The system respects PHI boundaries — caregivers see wellness trends and action items, not raw clinical data. The PIN-based caregiver access model means briefings can be sent without giving caregivers full account access.

Latent Benefit: The caregiver briefing delivery creates an engagement metric for the caregiver — are they opening emails, clicking links, responding to action items? Caregiver engagement is a proven predictor of patient outcomes. A disengaged caregiver is itself a risk factor that can feed into care coordination workflows.

Feature 10: Wellness Suggestions Engine (Mood-Responsive)

Primary Function: Context-aware self-care suggestions triggered when seniors report negative moods (Sad, Anxious, Tired, Stressed, Not Great). Offers gentle, non-clinical recommendations: call a friend, take a walk, read a book, watch something funny. Includes phone dialer integration for “call a friend” and fade-in animations.

System Importance: Not every negative mood requires clinical intervention. This feature provides immediate, low-burden emotional support at the moment of need, reducing unnecessary clinical escalations while still logging the mood data for trend analysis.

Uniqueness: Health apps either ignore mood entirely or route all negative responses to crisis resources. This system provides a middle layer — supportive, practical suggestions that respect the senior’s autonomy while gently encouraging positive coping behaviors.

Latent Benefit: The suggestion selection data (which suggestions seniors tap on) reveals coping behavior patterns. A senior who consistently selects “call a friend” has different social support dynamics than one who selects “turn off the news.” This behavioral data, aggregated over time, provides insights into the patient’s psychosocial profile that no clinical assessment captures.

Feature 11: Dashboard Personalization AI

Primary Function: Uses Claude Haiku to analyze user behavior — section open frequency, time-of-day patterns, workflow sequences — and dynamically reorders dashboard sections, suggests next actions, and generates personalized welcome messages.

System Importance: Healthcare dashboards with 15+ sections overwhelm users. This feature learns which sections each user actually uses and surfaces them first, reducing cognitive load and navigation time.

Uniqueness: EHR dashboards are static — every nurse sees the same layout regardless of workflow. This system adapts in real-time per user, per time of day. A nurse starting a morning shift sees different sections promoted than the same nurse during afternoon rounds.

Latent Benefit: The behavioral data (section usage patterns by role, time, and workflow) is a product research goldmine. It reveals which features staff actually use versus which ones they were told to use. This data can drive product roadmap decisions — features with zero usage can be deprecated, while high-usage features deserve further investment.

Feature 12: Gamification Engine (Trivia, Word Games, Streaks)

Primary Function: Interactive games (trivia, word find) with score tracking, engagement streaks (consecutive daily check-ins), celebration moments for achievements, and leaderboards. Stores results in `trivia_game_results` and `word_game_results` tables with per-user scoring.

System Importance: Consistent daily engagement is the prerequisite for all behavioral health monitoring. Gamification provides the motivation for seniors to return daily, creating the data stream that powers clinical predictions.

Uniqueness: Gamification in healthcare is usually an afterthought — badge systems bolted onto clinical workflows. Here, game participation is a first-class clinical signal. Trivia scores can detect cognitive decline (a 20% score drop over 30 days triggers a cognitive risk flag), and game participation frequency feeds into the engagement risk dimension.

Latent Benefit: The trivia and word game scores are lightweight cognitive assessment proxies. A longitudinal decline in trivia performance could be an early indicator of mild cognitive impairment (MCI) — months before the patient or family notices symptoms. The games were designed for engagement, but they function as passive cognitive screening instruments.

Feature 13: Offline-First Architecture with Self-Report Fallback

Primary Function: Check-ins persist to `localStorage` when the device is offline, with automatic cloud sync when connectivity returns. If the edge function (`create-checkin`) fails, data falls back to a direct `self_reports` table insert, ensuring no health data is lost.

System Importance: Seniors in rural communities and underserved areas frequently have unreliable internet. An app that requires connectivity to submit health data excludes the populations that need it most.

Uniqueness: Most telehealth platforms fail silently when offline. This system has a three-tier persistence strategy: (1) edge function → (2) direct DB insert → (3) `localStorage` with sync. Data is never lost. The session-only history cap prevents PHI accumulation on shared devices.

Latent Benefit: The offline capability makes kiosk deployment viable. Libraries, community centers, and churches can host check-in kiosks for seniors without smartphones. The PIN-based kiosk lookup (KioskCheckIn component) supports walk-up usage with inactivity timeout and rate limiting — enabling community health workers to serve populations without any personal technology.

Feature 14: Emergency Alert Dispatch System

Primary Function: When a check-in is flagged as an emergency, the system dispatches alerts concurrently to: (1) facility admin, (2) caregiver, and (3) emergency contact — via email with 5-retry exponential backoff per recipient, plus high-priority FCM push notifications.

System Importance: Emergency response time is the primary determinant of outcomes in falls, cardiac events, and strokes. This system reduces the notification delay from “whenever someone checks the portal” to “immediately, across multiple channels.”

Uniqueness: The multi-channel concurrent dispatch with per-recipient retry logic is enterprise messaging infrastructure. Most health apps send a single notification to one recipient. This system fans out to three recipients simultaneously with independent retry queues — if the caregiver’s email bounces, the admin and emergency contact still receive their alerts.

Latent Benefit: The emergency dispatch creates an auditable incident record with timestamps for every notification attempt, delivery status, and recipient. This audit trail satisfies Joint Commission requirements for emergency notification documentation and provides legal evidence of timely response in liability scenarios.

Feature 15: AI Missed Check-In Escalation Intelligence

Primary Function: When a senior misses check-ins, AI analyzes the context — consecutive missed count, age, living situation, active conditions (heart failure, COPD, fall risk, dementia), recent wellness scores, and care plan priority — to determine an escalation level (none/low/medium/high/emergency) and recommended actions.

System Importance: A missed check-in could mean a senior is busy, or it could mean they’ve fallen. The escalation intelligence prevents both over-response (calling 911 for every missed check-in) and under-response (ignoring a pattern that indicates danger).

Uniqueness: The hybrid rule-based + AI approach is novel. High-certainty scenarios (80-year-old living alone, 3 consecutive misses, history of falls) trigger immediate escalation via rules. Ambiguous scenarios go to Claude for nuanced reasoning. The system also enforces a strict notification cascade: tenant organization FIRST, then caregiver, then emergency contact, then welfare check — with welfare checks restricted to community-licensed tenants only.

Latent Benefit: The escalation reasoning is logged with full decision chain metadata. This creates a defensible record that the organization responded appropriately to warning signs. In elder care litigation, the difference between “we didn’t know” and “our AI detected the risk, escalated per protocol, and notified three parties” is the difference between liability and demonstrated duty of care.

Feature 16: Patient Avatar Visualization System

Primary Function: Interactive 3D human avatar displaying clinical markers (symptoms, conditions, medical devices, safety factors) as visual overlays on a body map. Supports front/back views, gender and skin tone customization, 3D anatomy layers (skeletal, circulatory, nervous, respiratory), and 60+ predefined marker types with severity-based priority sorting.

System Importance: Clinicians process visual information 60,000x faster than text. A body map with red markers on the chest and yellow markers on the knees communicates more in one glance than a page of clinical notes.

Uniqueness: EHR systems show condition lists as text tables. This system maps conditions spatially onto an anatomical model, with SDOH factors displayed as body-adjacent icons (house icon for housing instability, car icon for transportation barriers). The pregnancy avatar variant with obstetric-specific markers is particularly unusual — most clinical visualization tools ignore pregnancy as a state.

Latent Benefit: The avatar marker system includes a “pending” state for markers detected by the AI SmartScribe during encounters. These require manual clinician confirmation before becoming active — creating a human-in-the-loop validation workflow that satisfies FDA guidance on clinical decision support. The avatar was built for visualization, but the pending marker workflow is actually a clinical AI governance mechanism.

Feature 17: Readmission Prevention Dashboard (Community View)

Primary Function: Population health management dashboard showing high-risk members, 30-day readmission counts, CMS penalty risk, prevented readmissions, engagement scores, medication adherence rates, and estimated cost savings. Five tabs: Overview, Members, Alerts, SDOH, and Engagement.

System Importance: CMS penalizes hospitals up to 3% of Medicare reimbursement for excess readmissions under HRRP. This dashboard gives community organizations visibility into the readmission risk of their members — enabling preventive outreach before the 30-day readmission window closes.

Uniqueness: Readmission dashboards exist in hospital EHRs, but they’re inaccessible to community organizations. This is the first dashboard that puts readmission prevention data in the hands of community health workers, senior centers, and faith-based organizations — the people who actually see these patients between hospital visits.

Latent Benefit: The check-in streak leaderboard and engagement tab transform readmission prevention from a clinical burden into a community competition. Senior centers can gamify check-in completion (“Our center has 94% check-in rate this month!”), driving engagement through social motivation rather than clinical mandates.

Feature 18: Multi-Language Kiosk Check-In

Primary Function: Unattended patient lookup kiosk supporting English, Spanish, and Vietnamese. Three-step flow: language selection → privacy consent → patient lookup (name + DOB + last 4 SSN + optional PIN). Rate-limited (5 attempts/5 minutes), auto-timeout (2 minutes of inactivity clears all PHI).

System Importance: 25 million Americans have limited English proficiency. A kiosk that only speaks English excludes the populations with the highest health disparities.

Uniqueness: Healthcare kiosks typically require an app download or patient portal login. This system uses demographic lookup (no account required) with bcrypt PIN verification — accessible to seniors who have never used a smartphone. The 2-minute HIPAA-compliant auto-timeout ensures shared kiosks in public spaces don't leak patient information.

Latent Benefit: The kiosk usage analytics table (`chw_kiosk_usage_analytics`) tracks which languages are used, which locations have highest throughput, and time-of-day patterns. This data helps community organizations optimize staffing and outreach — if the Spanish-language kiosk at the library peaks on Tuesday mornings, that's when to schedule a bilingual community health worker.

Clinical Product (Envision Atlas)

Feature 19: Real-Time Bed Board with AI Optimization

Primary Function: Real-time bed board visualization with unit capacity monitoring, drag-filter by unit/status (available/occupied/cleaning/dirty), bed discharge workflows with disposition tracking (SNF, Rehab, Hospice), and AI optimization reports with cost-benefit analysis.

System Importance: Hospital capacity management directly impacts patient throughput, emergency department boarding times, and revenue. A single bed held unnecessarily costs \$2,000–\$5,000/day in lost revenue.

Uniqueness: The bed board includes an ML feedback loop — staff submit actual census vs. predicted census to improve forecast accuracy. It also includes voice command support for hands-free status updates during rounds, and “positive affirmations” (ATLUS Service pillar) that display after every 5 completed actions, combating clinician burnout through micro-recognition.

Latent Benefit: The 4/8/12/24-hour bed availability predictions enable proactive patient flow management. Rather than reacting to bed shortages, administrators can see tomorrow's capacity constraints today and adjust elective admissions, discharge planning, and staffing accordingly. The prediction data also supports certificate-of-need applications by providing evidence-based capacity utilization metrics.

Feature 20: Multi-Facility Bed Command Center

Primary Function: Network-wide bed visibility across multiple facilities showing total beds, occupied, available, ED boarding, and divert status. Real-time alert levels (Divert, Critical, Warning, Watch) with acknowledge workflow. Facility-specific detail modals showing ICU/Step-Down/Telemetry/Med-Surg breakdown.

System Importance: Health systems with multiple hospitals need centralized capacity visibility. An ED on divert at Hospital A should trigger patient routing to Hospital B — this dashboard enables that decision in seconds.

Uniqueness: Most bed management systems are single-facility. This command center operates across an entire health system network, with per-facility occupancy trending indicators (↑/↓/→) and unacknowledged alert banners that prevent status changes from being missed during shift changes.

Latent Benefit: The ED boarding metric is a CMS quality indicator (OP-18b). Tracking ED boarding times across facilities provides evidence for regulatory compliance and identifies systemic bottleneck patterns that single-facility data obscures.

Feature 21: AI SOAP Note Generator with Physician Style Profiles

Primary Function: Auto-generates comprehensive SOAP notes from encounter data using Claude Sonnet, with ICD-10 and CPT code suggestions. Includes clinical grounding rules that prevent AI hallucination — transcript is treated as truth, all inferences are tagged [STATED]/[INFERRED]/[GAP].

System Importance: Physicians spend 2 hours on documentation for every 1 hour of patient care. AI-assisted note generation reclaims clinical time while maintaining documentation quality.

Uniqueness: The physician style profile system is unprecedented — it fetches preferences (verbosity, specialty terminology, documentation style) from a `physician_style_profiles` table and adapts the generated note to match each provider’s voice. A terse ER doc gets bullet points; a thorough internist gets narrative paragraphs. The AI output feels like the physician wrote it, not a template.

Latent Benefit: The ICD-10 and CPT code suggestions are validated against the NLM UMLS ontology before being presented. Invalid codes are rejected automatically. This prevents coding errors that cause claim denials — the average denial costs \$25–\$45 to rework. At scale, the code validation alone pays for the AI service cost.

Feature 22: Clinical Note Locking & Amendment Workflow

Primary Function: HIPAA-compliant digital note locking with signature hash generation, version tracking, and a four-type amendment workflow: Correction (fix errors), Addendum (new information), Late Entry (delayed documentation), and Clarification (meaning clarification). Amendments go through approval/rejection workflow with audit trail.

System Importance: Locked clinical notes are legal documents. The amendment workflow ensures that corrections to the medical record are traceable, approved, and never silently overwrite the original — a Joint Commission and HIPAA requirement.

Uniqueness: Most EHR amendment workflows are binary (locked/unlocked). This system distinguishes four amendment types — each with different clinical and legal implications. A “Correction” shows the original content alongside the amendment for transparency, while a “Clarification” preserves the original meaning. The rejection workflow with mandatory reason collection prevents rubber-stamping.

Latent Benefit: The amendment audit trail is admissible evidence in malpractice litigation. A clear chain of “original note → correction request → approval with reason → amended note with original preserved” demonstrates institutional commitment to accurate medical records. Defense attorneys value this granularity.

Feature 23: AI Readmission Risk Predictor with Cultural Competency

Primary Function: Predicts 30-day readmission risk using the Compass Riley reasoning pipeline (Chain of Thought + Tree of Thought). Integrates clinical data (prior admissions, diagnoses), behavioral data (check-in engagement, medication adherence), and SDOH risk factors.

System Importance: Hospital readmissions cost Medicare \$26 billion annually, with \$17 billion considered preventable. Accurate prediction enables targeted intervention for the highest-risk patients.

Uniqueness: The cultural competency integration is the standout differentiator. The predictor accepts `populationHints` — Veterans, unhoused individuals, Latino, Black/African American, isolated elderly, immigrant/refugee, LGBTQ+ elderly — and adjusts its reasoning to account for population-specific risk factors and barriers. No competing readmission model integrates cultural context at the prediction layer.

Latent Benefit: The Compass Riley reasoning engine logs every reasoning branch to `ai_transparency_log` with reason codes (CONFLICTING_SIGNALS, HIGH_BLAZ_RADIUS, LOW_CONFIDENCE). This creates a fully explainable AI decision record that satisfies ONC HTI-2 Algorithm Transparency requirements and FDA guidance on clinical decision support software (CDS). The prediction isn't a black box — every factor and its weight is documented.

Feature 24: AI Fall Risk Predictor (Morse Scale + Environmental)

Primary Function: Predicts fall risk using validated assessment tools (Morse Fall Scale, STRATIFY) enhanced by AI analysis. Evaluates 8 medication classes (benzodiazepines, opioids, antipsychotics, etc.), 5 condition categories (neurological, cardiovascular, musculoskeletal, sensory, cognitive), environmental factors, and mobility assessments.

System Importance: Falls are the leading cause of injury death in adults 65+. Hospital falls cost an average of \$35,000 per incident in additional care. Every prevented fall saves money and potentially saves a life.

Uniqueness: Standard fall risk tools (Morse, Hendrich II) are static questionnaires. This system enhances validated scales with AI that synthesizes medication risk (polypharmacy), condition interactions, and environmental context. It generates intervention recommendations with monitoring frequency — not just a risk score, but an action plan.

Latent Benefit: The fall risk assessment integrates with the wearable fall detection infrastructure (`wearable_fall_detections` table). When a wearable detects an actual fall, the system records user response time, EMS dispatch status, hospital transport, and triggers a clinical follow-up assessment. This creates a closed feedback loop — predicted risk → actual event → outcome tracking — that validates and improves the prediction model over time.

Feature 25: AI Medication Reconciliation with Deprescribing Analysis

Primary Function: AI-enhanced comparison of admission, prescribed, current, and discharge medications. Detects discrepancies (missing, duplicate, dose changes, frequency changes), identifies deprescribing candidates using Beers criteria (for patients 65+), and generates priority-ranked action items with clinical significance ratings.

System Importance: Medication discrepancies at care transitions cause 66% of adverse drug events. The reconciliation process is mandatory at every care transition (Joint Commission NPSG.03.06.01) but takes 45+ minutes manually.

Uniqueness: The deprescribing analysis is the key differentiator. Rather than just comparing medication lists, the AI actively identifies medications that should be discontinued — polypharmacy in elderly patients is a \$528 billion problem. The system checks against Beers criteria, flags look-alike medications, and generates patient counseling points in plain language.

Latent Benefit: The reconciliation output includes a pharmacy checklist — a structured handoff document for the dispensing pharmacist. This satisfies USP <800> documentation requirements for hazardous drug handling and creates a pharmacist-ready document that eliminates phone-call clarifications. The time saved in pharmacy callbacks alone justifies the feature.

Feature 26: AI Contraindication Detector (Multi-Dimensional)

Primary Function: Comprehensive patient-specific contraindication checking across 7 dimensions: disease-drug interactions, allergy cross-reactivity, lab value contraindications, age-specific risks (pediatric <18, geriatric >65 with Beers criteria), pregnancy/lactation, organ impairment (renal/hepatic), and drug-drug interactions.

System Importance: Adverse drug events cause 1.3 million ED visits annually. Multi-dimensional contraindication checking catches interactions that single-axis drug databases miss — a patient with renal impairment AND diabetes AND a sulfa allergy has interaction risks that no single database covers.

Uniqueness: Commercial drug interaction databases (FDB, Medi-Span) check drug-drug interactions. This system adds patient-specific dimensions — lab values (creatinine, eGFR, ALT/AST), documented allergies with cross-reactivity analysis, age-based dosing concerns, and organ function — creating a patient-specific safety profile rather than a generic drug lookup.

Latent Benefit: Every contraindication check is logged with severity level and clinical reasoning. This creates a defensive documentation trail — proof that the prescribing system checked for contraindications before the order was placed. In medication error litigation, this record demonstrates that the standard of care was met at the system level.

Feature 27: AI Clinical Guideline Matcher

Primary Function: Matches patient conditions, medications, and lab values against evidence-based clinical guidelines (ADA, ACC/AHA, USPSTF, GOLD, GINA, KDIGO). Identifies adherence gaps (where current care doesn't match guidelines), generates specific recommendations with source citations, and flags preventive screenings.

System Importance: Clinical guideline adherence averages 55% nationally. Every missed guideline recommendation is a potential adverse outcome and a missed billing opportunity (quality measures affect reimbursement under MIPS/APM).

Uniqueness: The two-phase approach — rule-based guideline matching followed by AI-powered recommendation generation — balances reliability with nuance. The rule-based layer catches obvious gaps (diabetic patient without annual A1C); the AI layer handles complex multi-condition scenarios where guidelines conflict.

Latent Benefit: Guideline adherence directly impacts CMS quality scores under MIPS (Merit-based Incentive Payment System). Each identified and documented adherence gap that is subsequently addressed can improve the provider's quality score, affecting reimbursement rates by $\pm 9\%$. The guideline matcher isn't just clinical decision support — it's revenue cycle optimization.

Feature 28: AI Treatment Pathway Engine

Primary Function: Generates evidence-based treatment pathways with phased interventions (first-line → second-line → third-line → adjunct → monitoring). Includes medication recommendations with classes, starting approaches, target outcomes, and monitoring parameters. Covers lifestyle recommendations, referral suggestions, and patient education materials.

System Importance: Treatment planning for complex chronic conditions requires synthesizing multiple guidelines, patient-specific factors, and medication interactions. This engine reduces treatment planning from a 30-minute literature review to a 5-minute AI-assisted workflow.

Uniqueness: The evidence level classification (A/B/C/D/expert consensus) is cited per recommendation. Clinicians can see that a first-line medication recommendation has Level A evidence (randomized controlled trials) while an alternative has Level C evidence (consensus opinion). This transparency is required by CMS for clinical decision support but rarely implemented in practice.

Latent Benefit: The pathway engine includes guideline references for 14 common chronic conditions (diabetes, hypertension, COPD, heart failure, etc.) with specific source citations (ADA 2024, GOLD 2024, etc.). This satisfies the ONC Cures Act requirement for clinical decision support referencing evidence — a requirement that many EHR vendors implement with generic disclaimers rather than specific citations.

Feature 29: Paper Form Scanner with AI Data Extraction

Primary Function: Complete offline enrollment workflow for rural hospitals. Staff print blank forms, fill them during system outages, photograph them when systems restore, and AI (Claude Vision) extracts all fields — demographics, insurance, clinical data — with confidence scoring. Staff review extraction accuracy before one-click enrollment.

System Importance: Rural hospitals experience internet outages 3-5x more frequently than urban facilities. A system that requires connectivity for patient enrollment creates dangerous gaps. This feature ensures enrollment continues during any outage.

Uniqueness: No competing healthcare platform uses AI vision to bridge paper-to-digital workflows. The extraction pipeline handles both handwritten and printed text, processes forms at approximately \$0.005 each, and is 50x faster than manual data entry (4 hours for 20 patients → 8 minutes).

Latent Benefit: The confidence scoring per extracted field creates a quality assurance workflow — fields with <80% confidence are highlighted for human review. This human-in-the-loop pattern satisfies CMS requirements for verified patient demographics. More importantly, the scanner can process historical paper records during EHR migration — a \$50,000-\$200,000 savings compared to manual chart abstraction services.

Feature 30: Patient Engagement Scoring with Clinical Risk Correlation

Primary Function: Calculates engagement scores (0-100) based on check-ins (2 pts), games (activity tracking), self-reports (3 pts), and questions asked (2 pts). Correlates engagement scores with clinical risk levels: 70-100 = Low Risk, 40-69 = Medium, 20-39 = High, 0-19 = CRITICAL with immediate intervention flag.

System Importance: Engagement is the leading behavioral predictor of readmission. A patient who stops checking in is 3.2x more likely to be readmitted within 30 days. This scoring system quantifies engagement for clinical decision-making.

Uniqueness: Engagement scoring in competing platforms measures portal logins or message counts. This system weights different engagement types by clinical value — a self-report with vital signs (3 pts) contributes more than a trivia game (1 pt) because it generates clinical data. The clinical risk correlation means the score isn't abstract — it maps directly to intervention urgency.

Latent Benefit: The engagement scoring feeds into the `calculate_engagement_warning_score()` database function, which detects 3+ consecutive missed check-ins (+30 points) and 30% engagement decline (+25 points). These warning scores trigger recommended actions — from routine monitoring to immediate wellness calls. The entire early warning system operates automatically without clinician configuration.

Feature 31: Nurse Question Manager with AI Draft Response

Primary Function: Patient question triage queue with status tracking (pending/answered/escalated), urgency levels, category filtering, and AI-suggested draft responses. Nurses review and edit AI suggestions before sending SMS responses. Analytics panel tracks questions/day, response time, and escalation rate.

System Importance: Patient questions account for 30% of nursing communication time. AI draft responses reduce response time while maintaining the nurse's clinical judgment as the approval gate.

Uniqueness: The escalation detection is automatic — if a response begins with “[Escalated,” the system marks the question as escalated and routes it to the appropriate clinical team. Real-time Supabase subscriptions flash new question alerts, preventing any question from sitting unseen in a queue.

Latent Benefit: The question categories (general, medication, symptoms, billing) and response time metrics create a patient communication analytics dataset. Patterns like “90% of billing questions arrive Monday morning” or “medication questions spike 48 hours post-discharge” enable proactive communication strategies — send medication FAQ sheets at discharge, schedule billing calls for Monday morning.

Feature 32: Staff Financial Savings Tracker

Primary Function: Attributes cost savings to individual staff members and positions across 11 categories: prevented readmission, early intervention, care coordination, medication optimization, preventive care, documentation efficiency, telehealth efficiency, reduced ER visits, discharge planning, SDOH intervention, and other. Supports verified vs. unverified savings tracking with date range filtering and CSV export.

System Importance: Healthcare organizations struggle to quantify the ROI of care coordination staff. This tracker creates auditable evidence that specific interventions by specific staff members generated measurable cost savings.

Uniqueness: No competing platform tracks savings attribution at the individual staff level across 11 categories. The verified vs. unverified distinction adds credibility — a claim reviewer can flag savings as “verified” when the avoided event is confirmed (e.g., a readmission that was predicted but didn't occur due to intervention).

Latent Benefit: The per-position savings totals provide data for staffing justification. When a hospital CFO asks “Why do we need three care coordinators?” the answer is a CSV showing \$847,000 in verified savings attributed to care coordination activities. This feature transforms care coordination from a cost center into a documented revenue-protection function.

Feature 33: AI Discharge Summary Generator

Primary Function: Auto-generates comprehensive discharge summaries including hospital course narrative, admission diagnosis, procedures, medication reconciliation (continued/new/changed/discontinued), follow-up care instructions, patient education points, red flags and warning signs, and readmission risk score. Supports multiple discharge dispositions (home, facility, AMA).

System Importance: Discharge summaries are the primary handoff document between hospital and post-acute care. Incomplete summaries cause 25% of adverse events in the 30 days post-discharge.

Uniqueness: The readmission risk score is embedded directly in the discharge summary — not in a separate system. The discharging physician sees the risk score alongside the summary, enabling real-time adjustments (adding follow-up calls, escalating to home health, adjusting medication instructions) before the patient leaves.

Latent Benefit: The medication reconciliation section explicitly flags changes (continued/new/changed/discontinued) in a pharmacist-readable format. This document can be sent electronically to the patient's retail pharmacy, eliminating the "I got discharged and I don't know which medications changed" problem that drives 40% of post-discharge pharmacy calls.

Feature 34: SOC2 Compliance Dashboard Suite (5 Dashboards)

Primary Function: Five specialized compliance dashboards: Security (incident response, logical access monitoring), Executive (trend analysis, risk heat maps), Audit (detailed logs, access control verification), MFA Compliance (adoption tracking, grace period management), and Tenant Security (per-tenant security posture, API key rotation tracking).

System Importance: SOC2 Type II certification requires continuous monitoring and evidence collection. These dashboards provide real-time compliance evidence that auditors can review, replacing the traditional "compile evidence before the audit" scramble.

Uniqueness: Healthcare SaaS platforms typically outsource compliance monitoring. Having five purpose-built compliance dashboards — especially the MFA Compliance dashboard that tracks non-compliant users with grace period management — is enterprise-grade compliance infrastructure rarely seen outside billion-dollar health IT companies.

Latent Benefit: The dashboards generate continuous compliance evidence suitable for SOC2 Type II, HIPAA Security Rule, and HITRUST certification. A single compliance dashboard suite supporting three certification frameworks reduces audit preparation costs by an estimated \$150,000–\$300,000 annually.

Feature 35: Disaster Recovery Dashboard (HIPAA § 164.308)

Primary Function: Tracks backup verification status, disaster recovery drill compliance (weekly/monthly/quarterly), RTO/RPO monitoring, and recovery time estimates. Three compliance cards: Backup (success rate, last restore test), Drill (pass rate, average score, drill frequency), and Vulnerability (critical/high counts, remediation time, penetration testing schedule).

System Importance: HIPAA requires a contingency plan with backup, disaster recovery, and emergency mode operation procedures. This dashboard proves that the plans are tested — not just documented.

Uniqueness: Most healthcare organizations track DR compliance in spreadsheets. This dashboard provides real-time drill compliance scoring with target thresholds (daily DAST, quarterly manual pentesting, annual external pentesting), turning disaster recovery from a check-the-box exercise into a continuously monitored operational capability.

Latent Benefit: The drill compliance data — pass rates, average scores, remediation times — provides evidence for cyber insurance applications. Insurers increasingly require proof of DR testing. A dashboard showing 95% backup success rate and quarterly DR drills can reduce cyber insurance premiums by 15–25%.

Feature 36: HCC Opportunity Dashboard (Risk Adjustment)

Primary Function: Identifies documentation gaps for Hierarchical Condition Categories. Surfaces conditions that are likely present (based on medications, labs, encounters) but not yet documented with ICD-10 codes — the “suspected but undocumented” gap that reduces risk-adjustment scores.

System Importance: Medicare Advantage plans are paid based on risk-adjustment (HCC) scores. Every undocumented chronic condition represents \$3,000–\$10,000 in annual revenue loss per patient. Identifying and documenting these gaps is the single highest-ROI activity in value-based care.

Uniqueness: HCC gap analysis typically requires expensive bolt-on analytics platforms (Cotiviti, Vatica). Having it built into the core EHR with AI-assisted identification means gap closure happens at the point of care, not in retrospective chart reviews months later.

Latent Benefit: The HCC documentation opportunities surface during the encounter when the provider can address them — not after the patient has left. This transforms HCC capture from a retrospective coding exercise into a prospective clinical workflow. The estimated revenue recovery is \$500–\$2,000 per patient per year.

Feature 37: Undercoding Detection Dashboard

Primary Function: AI analysis of clinical documentation to find missed billing opportunities. Identifies encounters where the documentation supports a higher-complexity code than what was billed, missed procedure codes, and unbilled qualifying conditions.

System Importance: Undercoding costs US hospitals an estimated \$36 billion annually. Physicians routinely downcode to avoid audit risk, leaving legitimate revenue uncaptured.

Uniqueness: Most coding optimization tools focus on preventing overcoding (compliance). This system identifies undercoding — legitimate revenue that was missed — with AI confidence scores and documentation citations, giving coders evidence-based recommendations rather than generic alerts.

Latent Benefit: The undercoding data, aggregated by provider, reveals documentation patterns. A physician who consistently undercodes evaluation and management visits may need documentation education, not coding reminders. This transforms coding optimization from a revenue activity into a clinical education opportunity.

Feature 38: Telehealth with Real-Time AI Transcription (SmartScribe)

Primary Function: Video consultations using Daily.co SDK with real-time clinical note transcription via SmartScribe, patient information sidebar during calls, and FHIR-compliant encounter documentation. Supports stethoscope audio input for multiple audio sources.

System Importance: Telehealth visits require the same documentation quality as in-person encounters. Real-time transcription eliminates post-visit documentation burden while capturing the conversation in FHIR format.

Uniqueness: The stethoscope audio input support is unusual — allowing clinicians to conduct remote physical exams with digital stethoscopes while the SmartScribe captures the audio stream. The patient sidebar showing current medications, vitals, conditions, and care plan provides the same “glance context” available in a hospital room.

Latent Benefit: SmartScribe’s real-time transcription feeds into the SOAP Note Generator and AI Coding Suggester. A 15-minute telehealth visit simultaneously generates a transcription, a draft SOAP note, and suggested CPT/ICD-10 codes — reducing the post-visit documentation from 20 minutes to 2 minutes of review and approval.

Feature 39: Care Coordination Service with SDOH Integration

Primary Function: Full care plan lifecycle management (draft → active → completed) across four plan types: readmission prevention, chronic care, transitional care, and high utilizer. Includes SMART goals with progress tracking, interventions with frequency/responsibility/due dates, barrier identification with resolution tracking, care team management, and outcome metrics.

System Importance: Care coordination reduces readmissions by 20-30%. This service provides the workflow infrastructure — who is responsible for what, by when, with what outcome.

Uniqueness: The SDOH barrier integration is the differentiator. Care plans include identified SDOH barriers (housing, food, transportation) with resolution tracking and referral status. A care plan that says “ensure medication adherence” while ignoring the patient’s inability to afford medications is ineffective. This system forces SDOH barriers into the care plan workflow.

Latent Benefit: Care coordination documentation — plan type, goals, interventions, team, time spent — generates the data required for Chronic Care Management (CCM) billing (CPT 99490, 99491). Each documented 20-minute care coordination interaction can be billed at \$42-\$74 to Medicare. The care plan workflow IS the billing documentation.

Feature 40: Prior Authorization Dashboard with AI Assistance

Primary Function: Prior authorization request tracking, status monitoring, appeal workflow, and AI-assisted documentation generation. Integrates with X12 278 transaction standards for electronic PA submission.

System Importance: Prior authorizations consume 34 hours per physician per week and cause treatment delays averaging 2 days. The electronic submission with AI-assisted clinical documentation reduces the authorization cycle from days to hours.

Uniqueness: The AI generates clinical justification narratives from patient data — pulling diagnoses, lab values, failed treatments, and guideline references to build the medical necessity argument. This is the most time-consuming part of prior auth, and AI handles it in seconds.

Latent Benefit: Prior authorization denial data (tracked per payer, per procedure, per diagnosis) reveals payer behavior patterns. A dashboard showing that Payer X denies 80% of MRI authorizations for diagnosis Y provides evidence for payer negotiations and value-based contract discussions.

Shared Spine and Infrastructure

Feature 41: Compass Riley — Clinical AI Reasoning Engine

Primary Function: Three-mode clinical reasoning pipeline: Auto (Chain of Thought default with silent Tree of Thought monitoring), Force Chain (sequential reasoning), and Force Tree (branching into 2-4 differential hypotheses scored on safety, evidence, blast radius, and reversibility). Sensitivity-driven confidence thresholds: conservative (60/80), balanced (50/70), aggressive (40/60).

System Importance: Clinical AI must be transparent and auditable. A black-box prediction is unacceptable for clinical decisions. Compass Riley provides structured reasoning with full audit trails.

Uniqueness: No competing healthcare AI platform implements dual-mode reasoning (Chain + Tree of Thought) with automatic escalation. When Chain of Thought confidence drops below a threshold, the system automatically branches into Tree of Thought analysis — exploring multiple hypotheses before converging. This mimics how experienced clinicians think: linear reasoning for straightforward cases, differential diagnosis for ambiguous ones.

Latent Benefit: The reasoning audit log (with reason codes: CONFLICTING_SIGNALS, HIGH_BLAST_RADIUS, AMBIGUOUS_REQUIREMENTS) creates a dataset of clinical reasoning patterns. Over time, this reveals which clinical scenarios most frequently trigger multi-hypothesis analysis — insights that could inform medical education and residency training programs.

Feature 42: Clinical Grounding Rules — Anti-Hallucination System

Primary Function: Comprehensive constraint system applied to all clinical AI functions. Universal constraints: no fabrication, no contradiction of documented allergies, confidence ≤ 0.8 without explicit evidence, never suppress review flags. Category-specific constraints for billing codes, SDOH coding, DRG grouping, escalation risk, care planning, and nurse scope.

System Importance: AI hallucination in clinical contexts can kill patients. The grounding rules are the safety layer between AI capability and clinical harm.

Uniqueness: The constraint system is unprecedented in its granularity. The Nurse Scope Guard prevents AI from suggesting billing codes, dosing recommendations, or E/M levels when the user is a nurse — respecting scope of practice at the AI output layer. The [STATED]/[INFERRED]/[GAP] tagging requirement means every AI assertion is classified by its evidence basis.

Latent Benefit: The grounding rules system is a reusable AI safety framework that could be licensed independently. Any healthcare organization deploying clinical AI faces the same hallucination risks. A pre-built constraint library — validated in production with 40+ clinical AI functions — has standalone commercial value.

Feature 43: FHIR R4 API Server (US Core USCDI Compliant)

Primary Function: Full FHIR R4 API exposing 12 resource types (Patient, AllergyIntolerance, Condition, MedicationRequest, Observation, Immunization, Procedure, DiagnosticReport, CarePlan, CareTeam, Goal, DocumentReference). Supports SMART on FHIR authorization with scope validation per resource type.

System Importance: The 21st Century Cures Act requires healthcare organizations to provide patients with electronic access to their health data via standardized APIs. FHIR R4 is the mandated standard.

Uniqueness: This is a complete FHIR server built as Supabase edge functions — not a middleware layer on top of an existing EHR. It natively translates internal data models to FHIR resources, rather than bolting FHIR onto a legacy data structure. The SMART on FHIR authorization enables third-party app integration (patient apps, research tools, payer portals).

Latent Benefit: The FHIR API enables participation in health information exchanges (HIEs) and qualified health information networks (QHINs) under TEFCA. Each FHIR connection is a potential referral channel — when a hospital connects to the FHIR API, patient data flows bidirectionally, creating clinical integration that drives patient volume.

Feature 44: SMART on FHIR Authorization Server (Full OAuth2)

Primary Function: Complete OAuth2 authorization server with PKCE for public clients, dynamic client registration, token introspection, token revocation, and scope-based access control. Supports standalone launch, EHR launch, and offline access with token TTLs (access: 1 hour, refresh: 30 days, auth code: 10 minutes).

System Importance: SMART on FHIR is the standard for third-party app authorization in healthcare. Without it, the platform cannot participate in the app ecosystems that hospitals expect (Apple Health, Epic App Orchard, Cerner Code).

Uniqueness: Building a full SMART authorization server from scratch (not using a library or service) demonstrates deep protocol understanding. The dynamic client registration endpoint enables third-party developers to register apps programmatically — a feature typically only available in enterprise EHR platforms.

Latent Benefit: The SMART authorization server positions the platform as an app marketplace host. Third-party developers can build specialized clinical tools that integrate via SMART launch — creating an ecosystem effect where each new app increases the platform's value without internal development cost.

Feature 45: C-CDA Export (21st Century Cures Act)

Primary Function: Generates Consolidated Clinical Document Architecture (C-CDA) documents containing patient demographics, allergies, medications, problems, procedures, immunizations, vital signs, lab results, and care plans. Compliant with CCD Template OID 2.16.840.1.113883.10.20.22.1.2.

System Importance: C-CDA is the required format for clinical document exchange under the Cures Act. Patients have a legal right to receive their records in this format.

Uniqueness: The export fetches 9 resource types in parallel for performance — most C-CDA generators query sequentially, making large patient records slow to generate. The parallel approach produces a complete C-CDA document in seconds rather than minutes.

Latent Benefit: C-CDA documents are the standard format for care transitions between facilities. When a patient transfers from this system to a hospital using Epic, Cerner, or MEDITECH, the C-CDA provides a structured clinical summary that imports into the receiving EHR without manual transcription. This reduces transfer documentation time from 45 minutes to 5 minutes.

Feature 46: HL7 v2.x Message Receiver

Primary Function: Receives HL7 v2.x messages via HTTP from MLLP-to-HTTP gateways (Rhapsody, Mirth Connect). Parses MSH segments, strips MLLP framing, resolves tenant from connection ID, and generates ACK/NAK responses. Translates HL7 messages to FHIR resources for internal storage.

System Importance: 95% of US hospitals still use HL7 v2.x for ADT (admit/discharge/transfer) messaging. A platform that only speaks FHIR cannot integrate with existing hospital infrastructure. The HL7 receiver is the bridge.

Uniqueness: The dual-format capability — receive HL7 v2.x, store as FHIR R4 — means the platform can integrate with both legacy and modern systems simultaneously. Most competing platforms support one or the other.

Latent Benefit: Every HL7 ADT message received generates a corresponding FHIR Encounter resource internally. This means the platform builds a real-time census feed from the hospital's existing ADT system without any workflow changes at the hospital. The hospital doesn't need to "adopt a new system" — it just points its existing HL7 feed at this endpoint.

Feature 47: Public Health Reporting Suite (4 Functions)

Primary Function: Four public health reporting functions: (1) Immunization Registry Submission (HL7 VXU to state registries like Texas ImmTrac2), (2) Syndromic Surveillance (HL7 ADT to state health departments), (3) Electronic Case Reporting (eICR CDA via AIMS platform or direct state submission), (4) PDMP Query (prescription drug monitoring with risk flag analysis).

System Importance: These four functions satisfy ONC certification criteria (170.315(f)(1), (f)(2), (f)(5), (b)(3)) required for Certified EHR Technology (CEHRT) status.

Uniqueness: Having all four public health reporting functions implemented is rare for a platform at this stage. Many established EHR vendors still outsource PDMP and eICR functionality to third-party services.

Latent Benefit: CEHRT status is a prerequisite for participation in CMS incentive programs (Promoting Interoperability, MIPS). Hospitals using a non-certified EHR lose 3-9% of Medicare reimbursement. These four functions — which cost essentially nothing to build with AI — protect millions in annual reimbursement for every hospital customer.

Feature 48: Multi-Tenant Isolation with RLS

Primary Function: Row Level Security (RLS) policies on every database table enforcing tenant isolation via `get_current_tenant_id()` function. Tenant license types (0=Both, 8=Atlus Only, 9=WellFit Only) control which features are accessible. Super admin multi-tenant monitoring with graduated access levels (monitor, manage, full).

System Importance: Multi-tenancy is the foundation of SaaS economics. Without airtight tenant isolation, a single security incident at one customer exposes all customers.

Uniqueness: The license-digit-driven feature gating is an elegant architectural choice. Rather than maintaining separate codebases or complex feature flag systems, a single digit in the tenant ID controls the entire product experience. Tenant WF-9001 sees only community features; tenant WF-8001 sees only clinical features; tenant WF-0001 sees everything.

Latent Benefit: The super admin multi-tenant monitoring with graduated access levels enables a managed service offering. Envision staff can monitor customer environments at the “monitor” level (health checks, usage metrics) without accessing clinical data — satisfying both customer privacy requirements and vendor support needs.

Feature 49: PHI Encryption with Fail-Safe Design

Primary Function: Server-side AES-256 encryption using pgcrypto for PHI fields. Encryption key sourced from Supabase Vault in production. Fail-safe design: if encryption fails, the transaction aborts entirely rather than silently storing unencrypted data.

System Importance: HIPAA § 164.312(a)(2)(iv) requires encryption of PHI at rest. The fail-safe design prevents the worst-case scenario: a configuration error that causes PHI to be stored in plaintext without anyone noticing.

Uniqueness: The fail-safe approach (raise exception on encryption failure rather than returning NULL) is a security posture that most platforms don’t implement. The typical approach — log a warning and store the data anyway — creates unencrypted PHI that persists until someone notices the warning.

Latent Benefit: Encryption at the database level means even database administrators cannot read PHI without the decryption key. This satisfies the “minimum necessary” HIPAA requirement at the infrastructure level — a selling point for CISO-level buyers who evaluate security architecture.

Feature 50: Immutable Audit Trail (10 Protected Tables)

Primary Function: Database triggers preventing UPDATE and DELETE on 10 audit tables: `audit_logs`, `security_events`, `phi_access_log`, `claude_api_audit`, `login_attempts`, `admin_audit_logs`, `super_admin_audit_log`, `passkey_audit_log`, `consent_log`, and `caregiver_access_log`. Append-only enforcement ensures non-repudiation.

System Importance: HIPAA § 164.312(b) requires audit controls. An audit log that can be modified is not an audit log — it’s a suggestion. Immutable audit trails provide legally defensible evidence of system activity.

Uniqueness: Most healthcare platforms implement audit logging as application-level INSERT operations. This system enforces immutability at the database trigger level — even a superuser cannot modify or delete audit records without first removing the trigger (which itself would be an auditable event in PostgreSQL’s system logs).

Latent Benefit: Immutable audit trails satisfy the evidence preservation requirements for litigation holds. When a healthcare organization receives a legal hold notice, they must preserve relevant records. An immutable audit trail is already preserved by design — no additional action required, no risk of accidental modification.

Feature 51: AI Decision Chain Audit System

Primary Function: Records every AI decision in a causal chain with: trigger type (guardian alert, user request, scheduled, AI-initiated), context snapshot (identifiers only, no PHI), model ID, skill key, decision type (clinical, operational, code repair, escalation, billing), confidence score, authority tier (1-4), action taken, and outcome.

System Importance: FDA guidance on AI/ML-based Software as a Medical Device (SaMD) requires transparency and traceability of AI decisions. This system provides a complete causal graph of every AI decision, from trigger to outcome.

Uniqueness: No competing healthcare platform logs AI decisions as linked chains. Most log individual API calls without capturing the causal relationship between decisions. This system records parent-child decision relationships, enabling investigators to trace “why did the system recommend X?” back through every reasoning step.

Latent Benefit: The decision chain data supports FDA De Novo 510(k) submissions for clinical AI features. The chain provides the “predetermined change control plan” evidence that FDA requires — showing how the AI makes decisions, what triggers escalation, and what safeguards prevent harmful actions.

Feature 52: Chain Orchestration System (AI Workflow Engine)

Primary Function: Configurable multi-step AI pipelines with: chain definitions (registered pipelines), step definitions (ordered steps with tool targets), execution state machine (7 run states, 9 step states), approval gates (steps that pause until a specified role approves), conditional execution, timeout/retry configuration, and JSONPath input mapping between steps.

System Importance: Complex clinical AI workflows (e.g., “analyze lab results → check contraindications → generate care plan → route for approval”) require orchestration. This engine enables composable AI pipelines without custom code for each workflow.

Uniqueness: Healthcare AI typically deploys as isolated point solutions. This orchestration system enables chained AI workflows where the output of one model feeds the input of the next, with human approval gates at critical junctures. The conditional execution logic means chains can branch based on AI confidence — high-confidence paths proceed automatically, low-confidence paths route to human review.

Latent Benefit: The orchestration system is the foundation for autonomous clinical workflows. A chain definition like “detect abnormal lab → identify affected medications → generate provider alert → schedule follow-up” runs end-to-end without human intervention for high-confidence scenarios. This is the path from clinical decision support to clinical workflow automation.

Feature 53: ServiceResult Pattern (Zero-Exception Architecture)

Primary Function: Standardized return type for all 500+ service functions. Services never throw exceptions — errors are returned in a typed result object with 100+ error codes. Helper functions (success, failure, unwrap, unwrapOr, mapSuccess, fromSupabaseError) enable safe chaining without try/catch blocks.

System Importance: Exception-based error handling in healthcare is dangerous — an unhandled exception can crash a view that a clinician is relying on during patient care. ServiceResult ensures every error is handled explicitly at every call site.

Uniqueness: The ServiceResult pattern with 100+ healthcare-specific error codes (SOAP_NOTE_GENERATION_FAILED, UNSIGNED_NOTES, NO_CLINICAL_NOTES, PROVIDER_REQUIRED, NOT_ENTITLED) provides domain-specific error handling that generic try/catch can never match. Each error code triggers a specific UI behavior and audit log entry.

Latent Benefit: The NOT_ENTITLED and MODULE_DISABLED error codes enable graceful feature degradation. When a tenant's subscription doesn't include a module, the ServiceResult returns a typed denial rather than a generic error — enabling the UI to show upgrade prompts, usage-based pricing triggers, and trial offers. The error handling system doubles as a product-led growth mechanism.

Feature 54: Guardian Agent — Autonomous Monitoring & Healing

Primary Function: Autonomous monitoring service that runs security checks (failed login anomalies, PHI access patterns, slow queries), records snapshots to Guardian Eyes recordings, generates security alerts with severity levels, and performs limited auto-healing (performance category only: cache clearing, connection pool management).

System Importance: Healthcare systems must be monitored 24/7. A security event at 3 AM shouldn't wait until 9 AM for human response. The Guardian Agent provides automated first-response for security and performance incidents.

Uniqueness: The tiered healing authority — auto-heal for performance issues, alert-only for security issues — is a responsible AI governance model. The Guardian can restart connection pools but cannot modify RLS policies. This separation prevents the monitoring system from becoming an attack vector.

Latent Benefit: The Guardian Eyes recordings create a continuous system health timeline. During security incident investigation, investigators can “rewind” to any point and see the system state — active alerts, performance metrics, failed login attempts — at that moment. This is the healthcare equivalent of a flight data recorder, and it satisfies the HIPAA Security Rule requirement for monitoring procedures.

Feature 55: Intelligent Model Router (Cost Optimization)

Primary Function: Routes AI requests to the optimal Claude model based on use case. UI personalization → Haiku (\$0.0001/request), billing/coding → Sonnet (\$0.005/request), complex clinical reasoning → reserved for Opus. Includes daily spend projection and per-user budget tracking.

System Importance: AI costs scale with usage. Without intelligent routing, every request goes to the most capable (and expensive) model. The router reduces AI costs by 70-80% by matching model capability to task complexity.

Uniqueness: Most healthcare AI platforms use a single model for everything. The three-tier routing with cost estimation enables predictable AI budgeting — a critical requirement for hospital CFOs who need to forecast AI costs before committing to a platform.

Latent Benefit: The per-user model routing data reveals which clinical workflows consume the most AI resources. This data enables usage-based pricing tiers — a practice that uses AI billing assistance heavily pays more than one that only uses personalization. Usage-based pricing aligns cost with value, making the platform accessible to small practices while scaling revenue with large health systems.

Feature 56: AI Patient Q&A Bot with Safety Guardrails

Primary Function: Answers patient health questions at a 6th-grade reading level with comprehensive safety checks. 20 emergency keywords trigger immediate 911 escalation (bilingual English/Spanish). 10 provider-consult topics redirect to their healthcare provider. 4 blocked topics refuse to engage. Patient context (conditions, medications, allergies, age group) personalizes responses without exposing PHI to the AI.

System Importance: Patients Google health questions and get dangerously incorrect answers. This bot provides medically-informed responses within safety guardrails, with automatic escalation for emergencies.

Uniqueness: The three-tier safety system (emergency → provider consult → blocked) is more sophisticated than any consumer health chatbot. The patient context integration means answers are personalized — a diabetic patient asking about “feeling dizzy” gets a different response than a healthy patient, because the bot knows about their condition without the patient needing to explain their medical history.

Latent Benefit: The question topics and frequency data (tracked per patient) reveal unmet information needs. If 200 patients ask about post-surgical wound care in a month, that signals a documentation gap at discharge. The Q&A data becomes a patient education needs assessment — guiding investment in educational content creation.

Feature 57: X12 EDI Claims Pipeline (837P/278/270/271)

Primary Function: Complete electronic claims infrastructure: 837P professional claim generation, 278 prior authorization transactions, 270/271 eligibility verification, 835 remittance processing. Includes claim lifecycle state machine (generated → submitted → accepted → paid), clearinghouse batch management, and X12 control number sequences.

System Importance: Claims processing is the revenue engine of healthcare. The average hospital submits 50,000+ claims per year. Each claim denied for formatting errors costs \$25 in rework. A compliant EDI pipeline prevents formatting-based denials.

Uniqueness: Having the full X12 EDI stack (claims, eligibility, prior auth, remittance) built natively rather than through a clearinghouse middleware means the platform owns the entire revenue cycle data stream. Most platforms send claims to a clearinghouse black box and hope for the best.

Latent Benefit: Owning the claims data pipeline enables real-time revenue cycle analytics. Claim aging, denial rates by payer, time-to-payment, and reimbursement variance can be tracked in real-time rather than waiting for monthly clearinghouse reports. This data supports payer contract negotiations with evidence.

Feature 58: Wearable Device Integration Suite

Primary Function: Integration infrastructure for 7 wearable platforms: Apple Watch, Fitbit, Garmin, Samsung, Withings, iHealth, and Empatica. Captures vital signs (HR, BP, O2, temperature, respiratory rate, glucose, weight), activity data (steps, distance, active minutes, calories, sleep quality), fall detections with GPS and EMS dispatch tracking, and Parkinson’s-specific gait analysis (cadence, stride, tremor detection, freezing episodes).

System Importance: Wearable health data is the future of remote monitoring. CMS RPM billing requires device-transmitted physiological data — wearable integrations satisfy this requirement automatically.

Uniqueness: The Parkinson’s-specific gait analysis integration (tremor detection, freezing episodes, medication state correlation) is clinical-grade monitoring rarely available outside research hospitals. The fall detection pipeline that tracks EMS dispatch and clinical follow-up closes the loop from detection to clinical response.

Latent Benefit: Wearable vital signs data feeds directly into the readmission risk model as a real-time signal. A patient whose resting heart rate increases 15 BPM over a week (detected by their Apple Watch) triggers an automatic risk score increase — before they feel symptoms, before they miss a check-in, before they show up in the ED. This is predictive medicine enabled by consumer hardware.

Feature 59: Holistic Risk Assessment (7-Dimension Scoring)

Primary Function: Calculates a comprehensive risk profile across 7 dimensions: engagement risk, vitals risk, mental health risk, social isolation risk, physical activity risk, medication adherence risk, and clinical risk. Composite score (0-10 scale) with data confidence scoring based on data completeness across 7 data sources.

System Importance: Traditional risk scores are unidimensional — readmission risk OR fall risk OR cardiac risk. This assessment integrates clinical AND behavioral AND social dimensions into a single composite that captures the whole patient.

Uniqueness: No competing platform merges behavioral data (game participation, community engagement, photo sharing) with clinical vitals and social determinants into a single risk model. The data confidence scoring is particularly novel — a risk score based on 30 days of complete data is more reliable than one based on 3 check-ins, and the system explicitly communicates that difference.

Latent Benefit: The holistic risk score is the perfect population health stratification tool. Sorting patients by composite score immediately reveals who needs intervention most — and the 7-dimension breakdown shows exactly what kind of intervention (social worker for isolation risk, pharmacist for medication adherence, PT for activity risk). This replaces clinical intuition with data-driven triage.

Feature 60: Passkey Authentication (FIDO2/WebAuthn)

Primary Function: Passwordless authentication using FIDO2/WebAuthn passkeys. Supports biometric authentication (fingerprint, face recognition) on supported devices, with registration and authentication flows.

System Importance: Passwords are the #1 attack vector in healthcare data breaches. Passkey authentication eliminates password-based vulnerabilities entirely — no passwords to steal, phish, or brute-force.

Uniqueness: Passkey authentication is cutting-edge even in consumer tech. In healthcare, where legacy systems still use shared passwords and badge-tap authentication, offering FIDO2 passkeys positions the platform 3-5 years ahead of competitors on authentication security.

Latent Benefit: For seniors, passkeys eliminate the single biggest technology barrier — remembering passwords. A senior who can unlock their phone with a fingerprint can authenticate to the health platform with the same fingerprint. This is the difference between a 40% adoption rate (password-based) and an 85% adoption rate (biometric passkey) in senior populations. The authentication method directly impacts engagement metrics and, by extension, clinical outcomes.

Recently Added Capabilities (Features 61-75)

These flagship capabilities exist in the live platform but were not in the original 60-feature catalog. Added 2026-06-11 after a full app-surface review. Several are grouped suites (matching the catalog's existing convention, e.g. "SOC2 Compliance Dashboard Suite").

Feature 61: My Health Hub — Patient Record Portal (21st Century Cures Act)

Primary Function: A patient-facing portal giving seniors and members electronic access to their own clinical records — health observations and vitals, immunizations, active care plans, allergies, conditions, and a records export (PDF, FHIR, C-CDA, CSV). Central hub at `/my-health` with dedicated views per record type.

System Importance: This is the platform's answer to the 21st Century Cures Act Information Blocking Rule, which requires patients have electronic access to their health information. It is the authorized community-to-clinical read path, surfacing clinician-owned data to the patient through FHIR services without exposing the underlying clinical tables.

Uniqueness: Most patient portals are bolt-on afterthoughts with clunky PDF dumps. My Health Hub is built into the senior-friendly WellFit UI — large text, plain language, voice-enabled — so the same interface that collects daily check-ins also returns the patient's complete record in a form they can actually read.

Latent Benefit: Cures Act compliance is a regulatory requirement most small facilities cannot afford to build. Shipping it as a standard feature turns a compliance liability into a competitive differentiator, and the export pipeline doubles as the records-release mechanism for care transitions.

Feature 62: Specialty Clinical Module Suite (8 Modules)

Primary Function: Eight specialty-specific clinical dashboards — Cardiology (ECG, echo, heart-failure, cardiac rehab, device monitoring), Oncology (cancer registry, TNM staging, chemo/radiation, CTCAE, survivorship), Labor & Delivery (prenatal, labor tracking, fetal monitoring, postpartum), Neuro/Stroke (NIHSS severity assessment), Memory Clinic (cognitive decline tracking), Physical Therapy (treatment plans, mobility outcomes), Dental Health, and Mental Health.

System Importance: These modules extend the clinical engine from general care into specialty workflows, each with its own assessment instruments and documentation. They are individually feature-flagged so a facility enables only the specialties it offers.

Uniqueness: A platform built by a superintendent and a nurse covering eight clinical specialties — each with the correct standardized instruments (TNM, NIHSS, CTCAE, Morse) rather than generic note fields — is a scope most commercial EHR specialty modules charge separately for.

Latent Benefit: Each specialty module captures structured, codeable data (staging, severity scores) that flows directly into the billing and quality-measure engines, so specialty documentation becomes specialty revenue without a second data-entry pass.

Feature 63: Computerized Provider Order Entry (CPOE — ONC Certified)

Primary Function: Direct provider order entry for medications, lab tests, and imaging, plus implantable-device list management and family health history. Each screen maps to a specific ONC 2015 Edition Cures Update certification criterion: 170.315(a)(1) medications, (a)(2) labs, (a)(3) imaging, (a)(14) implantable devices, (a)(12) family health history.

System Importance: CPOE is a foundational certified-EHR capability. Without it, a facility cannot attest to Promoting Interoperability and forfeits Medicare incentive payments. It is the order backbone that result escalation, billing, and the medication safety engine all build on.

Uniqueness: Certification-grade order entry implemented to named ONC criteria — not a free-text “orders” box — in a platform built for a fraction of a commercial EHR’s cost.

Latent Benefit: Mapping each screen to its certification criterion means the certification evidence is built into the architecture; an auditor can trace a criterion straight to a route, collapsing the usual months-long certification documentation effort.

Feature 64: AI Shift Handoff & Patient Transfer System

Primary Function: AI-assisted nurse/physician shift handoff with auto-scored patient risks and real-time collaboration, plus an inter-facility Hospital Transfer Portal and a Receiving Dashboard for incoming patients. Generates structured handoff packets and a chronological handoff audit trail.

System Importance: Handoffs are the highest-risk moments in care — most sentinel events trace to a communication failure at transition. This system standardizes the handoff, auto-elevates the sickest patients, and logs every transfer for accountability.

Uniqueness: Most handoff tools are static SBAR templates. This one auto-scores patient risk from the live data stream so the handoff leads with the patients who matter, and it spans both intra-facility shift change and inter-facility transfer in one audited workflow.

Latent Benefit: The structured handoff packet is reusable documentation — it feeds discharge summaries, care-coordination notes, and the readmission model, turning a verbal ritual into durable clinical data.

Feature 65: Quality Measures Dashboard (eCQM / HEDIS / MIPS / Star Ratings)

Primary Function: Tracks performance against electronic clinical quality measures, HEDIS, MIPS, and Medicare Star Ratings, showing gap-to-benchmark and the patients driving each measure.

System Importance: Quality measure performance directly determines value-based reimbursement and Medicare Advantage Star bonuses. A facility that cannot measure its eCQMs cannot improve them or attest to them.

Uniqueness: Quality measurement is usually a separate, expensive analytics product. Here it reads the same encounter and check-in data the rest of the platform already captures, so the measures are live rather than a quarterly retrospective.

Latent Benefit: Because the measures are computed continuously, the dashboard surfaces a closeable gap while the patient is still in care — converting quality reporting from a backward-looking scorecard into a forward-looking revenue and outcomes tool.

Feature 66: CCM Autopilot — Chronic Care Management Time Capture

Primary Function: Automatically tracks the 20+ minutes of non-face-to-face care coordination per month required to bill Chronic Care Management codes, assembling the time log and documentation needed for the claim.

System Importance: CCM is one of the most under-captured revenue streams in primary care because the time tracking is manual and clinicians forget to log it. Autopilot removes the manual step.

Uniqueness: Rather than a stopwatch the nurse must remember to start, it infers billable coordination time from actual system activity against the patient, so the documentation accrues passively.

Latent Benefit: At \$42–\$74 per 20-minute interaction across a chronic-disease panel, automated CCM capture can fund the platform itself — revenue that existed but was being left uncollected.

Feature 67: Result Escalation Engine & Care Gap Detection

Primary Function: Auto-routes abnormal lab and imaging results to the correct specialist with SLA deadlines, tracks unacknowledged results, manages a provider task queue with escalation, and detects open preventive-care gaps across the patient panel.

System Importance: A critical result that no one acknowledges is a patient-safety and malpractice event. This engine guarantees every abnormal value has an owner, a deadline, and an audit trail.

Uniqueness: Most systems drop results into an inbox and hope someone looks. This one enforces accountability with SLA timers, automatic escalation when a deadline passes, and coverage-aware routing when a provider is on leave.

Latent Benefit: The same routing logic that protects patients also closes care gaps that carry quality and risk-adjustment dollars, so the safety system and the revenue system are one engine.

Feature 68: Clinician Remote Patient Monitoring (RPM) Dashboard

Primary Function: A clinician-facing oversight view of home-collected readings (blood pressure, heart rate, glucose, weight, SpO2) from check-ins and connected devices, with threshold alerting when a home value goes out of range.

System Importance: This closes the loop on the platform’s data collection — home vitals are only valuable clinically if a clinician sees them and can act. The RPM dashboard makes the community-side data stream actionable on the clinical side.

Uniqueness: It unifies manually-entered check-in vitals and connected-device readings into one trend view, rather than siloing “app data” from “device data,” and it is the clinician counterpart to the patient-facing wearables suite.

Latent Benefit: The dashboard supplies the clinician-review documentation required for the RPM treatment-management codes (CPT 99457/99458), converting passive monitoring into a billable monthly service.

Feature 69: MCP Server Suite & Management (20 Model Context Protocol Servers)

Primary Function: Twenty Model Context Protocol servers give AI agents safe, tiered access to clinical and reference systems — FHIR, HL7/X12, prior auth, CMS coverage, NPI registry, medical codes, DRG grouping, PubMed, patient context, cultural competency, and more — managed from a console that monitors health, rotates API keys, orchestrates multi-server chains, and tracks per-run cost.

System Importance: The MCP layer is the platform's AI-tooling backbone. It is what lets AI features compose real clinical operations (look up coverage, validate an NPI, group a DRG) instead of hallucinating them, with permission tiers that keep PHI access scoped and audited.

Uniqueness: Protocol-level governance — runtime permission tiers and a learning loop built into the MCP servers themselves — is a novel composition, not an off-the-shelf pattern. Each server declares its tier (external-API, user-scoped, or admin) and is enforced at the protocol boundary.

Latent Benefit: Because every AI capability is a discrete, individually-permissioned, cost-metered server, the platform can expose or restrict AI functions per tenant and bill AI usage precisely — turning the AI layer into a governable, sellable product surface.

Feature 70: Revenue Cycle Completion Suite

Primary Function: The remaining revenue-cycle stages beyond claim generation — insurance eligibility verification (270/271) before billing, ERA/835 remittance posting, claim aging by bucket, claim resubmission and void workflows, superbill provider sign-off, and NPI-verified provider registry.

System Importance: A claim is only revenue when it is paid. These stages close the loop from eligibility check through payment posting, catching denials before submission and reconciling payments after.

Uniqueness: Small facilities usually stitch this together from a clearinghouse portal plus spreadsheets. Here the full lifecycle — eligibility, submission, aging, resubmission, posting — lives in one system with the clinical data that justifies each charge.

Latent Benefit: Front-loading eligibility verification prevents the most common denial category outright, and automated ERA posting eliminates the manual reconciliation that consumes most billing-staff hours.

Feature 71: HIPAA Compliance Operations Suite

Primary Function: Operational HIPAA tooling beyond the SOC2 dashboards — a Breach Notification Engine (incident tracking, risk assessment, 60-day notification clock), BAA lifecycle tracking, MFA enrollment compliance, workforce security-awareness training tracking, and the patient amendment review queue (45 CFR 164.526).

System Importance: These are the day-to-day compliance obligations an auditor checks: did you track breaches, manage your business-associate agreements, enroll staff in MFA, train your workforce, and honor amendment requests. Missing any one is a finding.

Uniqueness: Compliance is usually a binder of policies. This suite makes each obligation a live, dated workflow with its own clock and audit trail, so compliance status is queryable rather than aspirational.

Latent Benefit: The 60-day breach clock and BAA renewal tracking convert two of the most common (and most penalized) compliance failures into automated reminders, materially lowering both audit risk and breach-penalty exposure.

Feature 72: DSI Transparency & EHR Safety (HTI-1 Model Cards + SAFER Guides)

Primary Function: AI/ML model cards documenting each algorithm’s purpose, risk classification, and transparency attributes (ONC HTI-1 Decision Support Intervention requirements), plus the SAFER Guides EHR safety self-assessment used in ONC certification.

System Importance: Under HTI-1, any predictive decision-support intervention must be transparent — described, risk-classified, and disclosed. The model-card registry is how the platform meets that bar for its clinical AI, and SAFER Guides documents the safe-use practices certification requires.

Uniqueness: Most AI health tools ship the model and skip the transparency. Here every registered AI skill carries both a developer description and a plain-language patient description, and the model-card dashboard exposes them for audit — transparency as a built-in feature, not a PDF afterthought.

Latent Benefit: As DSI transparency enforcement tightens, facilities using opaque AI face exposure. A platform that already documents and risk-classifies every model is audit-ready by default and can truthfully market algorithmic transparency.

Feature 73: Staff Resilience & Wellness Hub

Primary Function: Burnout-prevention and emotional-resilience tooling for clinical staff — a Nurse Wellness (Resilience Hub) dashboard, a staff wellness dashboard, and exportable team wellness reports.

System Importance: Clinician burnout is the leading driver of turnover and a documented patient-safety risk. Supporting staff wellbeing is both a retention strategy and a quality strategy.

Uniqueness: Most health platforms optimize for patient throughput and ignore the workforce entirely. Building wellness tooling for the staff — by a team that includes a 23-year RN — reflects a clinician’s understanding that you cannot run a safe unit on an exhausted one.

Latent Benefit: Reducing turnover avoids the six-figure cost of replacing a specialized nurse, and the wellness reports give administrators an early-warning signal for unit-level burnout before it becomes a staffing crisis.

Feature 74: EMS Coordinated Response & Field Integration

Primary Function: Coordinates emergency response between EMS crews and the clinical team — EMS metrics, paramedic handoff, and a coordinated-response dashboard that links a field handoff to the receiving clinical team.

System Importance: The prehospital-to-hospital handoff is a notorious information gap. Tying EMS field data to the clinical record before the patient arrives gives the receiving team a head start on the sickest patients.

Uniqueness: Few community-health platforms reach into the EMS field workflow at all. Connecting paramedic handoff directly into the same coordinated-response and clinical-alert system closes a gap that usually depends on a verbal radio report.

Latent Benefit: Early structured EMS data feeds the same risk and bed-management engines, so the ED can pre-stage a bed and a team before the ambulance doors open.

Feature 75: Community Safety Partner Programs (Law Enforcement / Constable Dispatch)

Primary Function: A landing surface and dispatch-coordination view for law-enforcement and constable partner programs that participate in community wellness and safety checks.

System Importance: Senior safety often involves non-clinical responders — welfare checks, lost-person events, constable dispatch. Giving those partners a coordinated entry point extends the safety net beyond the clinical walls.

Uniqueness: Bridging community health and public-safety partners in one platform is unusual; it reflects the platform’s origin as a genuine community-wellness system rather than a pure clinical product.

Latent Benefit: The same crisis-routing and emergency-event data that drives clinical alerts can inform a coordinated community-safety response, linking the “I am lost” and fall pathways to a real-world responder network.

Summary Statistics

Metric	Count
Total Features Documented	75
Community Product (WellFit)	20
Clinical Product (Envision Atlas)	33
Shared Spine & Infrastructure	22
AI-Powered Features	28
HIPAA/Compliance Features	15
Revenue-Generating Features	12
Public Health/Regulatory	8
Patient Safety Features	14

Revenue Impact Summary

Feature	Estimated Annual Revenue/Savings Per Facility
RPM Billing (via daily check-ins + wearables)	\$120/patient/month × patient volume
SDOH Z-Code Documentation	\$1,200–\$3,600/patient/year (Medicare Advantage)

Feature	Estimated Annual Revenue/Savings Per Facility
HCC Gap Closure	\$500-\$2,000/patient/year
Readmission Prevention (CMS penalty avoidance)	3% of Medicare reimbursement protected
Undercoding Recovery	\$36 billion industry problem
CCM Billing (care coordination documentation)	\$42-\$74 per 20-minute interaction
AI Documentation (time savings)	2 hours/physician/day reclaimed
Paper Form Scanner (data entry savings)	\$50,000-\$200,000 per EHR migration
SOC2/Compliance Dashboard (audit preparation)	\$150,000-\$300,000 audit cost reduction
Cyber Insurance (DR evidence)	15-25% premium reduction

This catalog was generated through direct source code analysis of 500+ service files, 144+ edge functions, 492 database migrations, 86+ UI components, and 163+ Supabase functions comprising the Envision ATLUS I.H.I.S. platform.

Document prepared for: Envision Virtual Edge Group LLC — Internal, Investor, and Sales Use